

Strategic Governance & Financial Realignment

Unlocking a \$200,000 Net Surplus for We Care



\$61,720
Net Output

The Default Model

Strategic
Realignment



\$205,720
Net Output

The Strategic Model

Mission Objective:

Secure sustainable funding to provide free outpatient medical treatment to needy patients.

The Pivot:

Moving from a physically capped standard event to a highly leveraged, multi-tier corporate fundraising model.

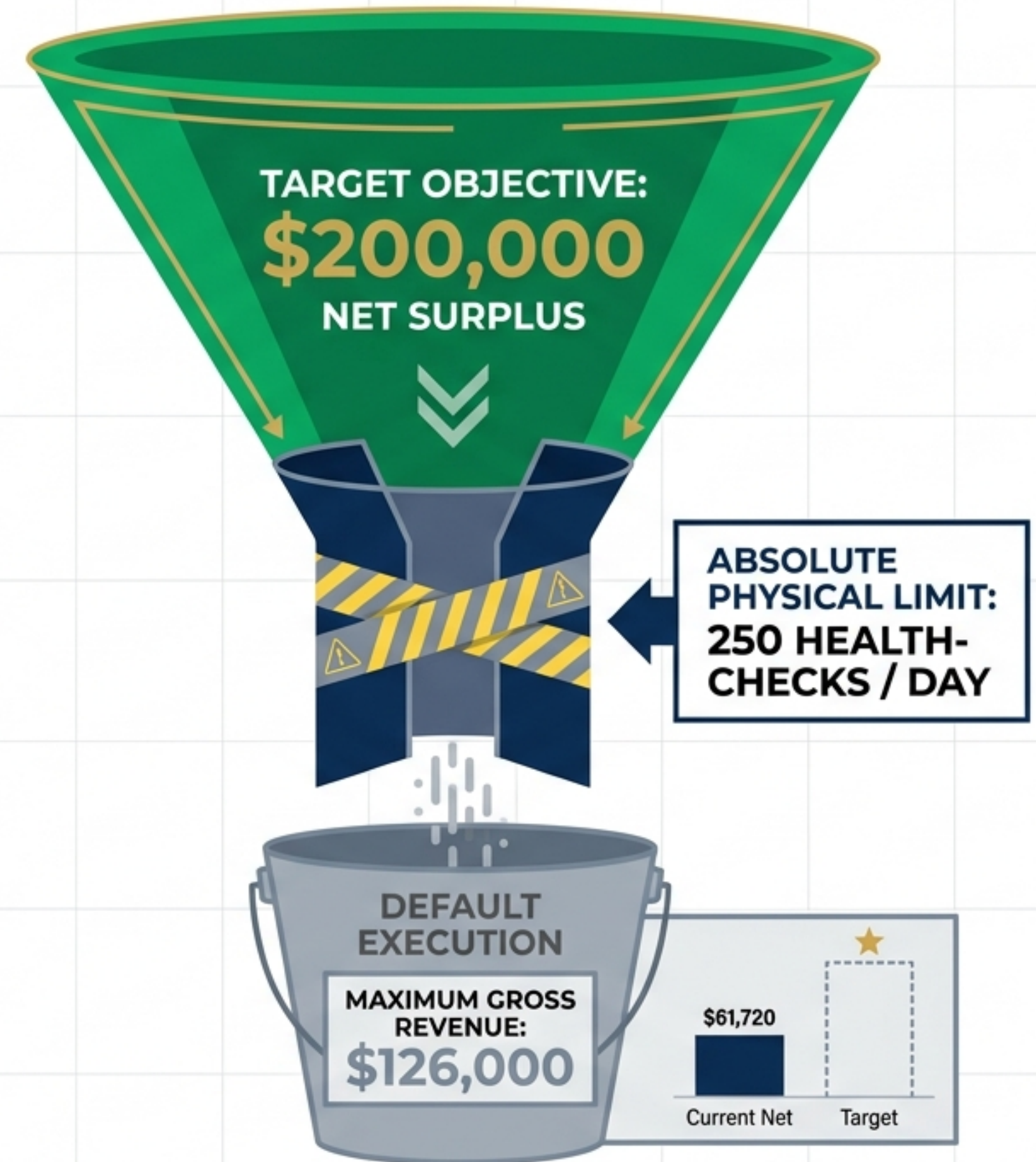
THE BLUEPRINT FOR REALIGNMENT

THE CONSTRAINT

We Care currently relies on two medical clinics and a single-day event structure. We cannot treat our way to our financial goal.

THE FAILURE POINT

A hard cap of 250 physical stations strictly limits standard ticket revenue, resulting in an expected net of just **\$61,720**—failing our target by over 69%.



THE BLUEPRINT FOR REALIGNMENT

The Turnaround Framework

How We Achieve the Strategic Model

Executive Summary: To hit our target, we must shift our operational paradigm. We will self-fund our risk, optimize our corporate pricing, and sever the link between revenue and physical medical check capacity.

Unlocking \$200,000+ Net Surplus



**Pillar 1:
Capital Realignment**

Repurpose idle cash reserves to internally fund all upfront overhead.



**Pillar 2:
Efficiency Recovery**

Slash our non-compliant fundraising cost ratio using structured corporate tiers.



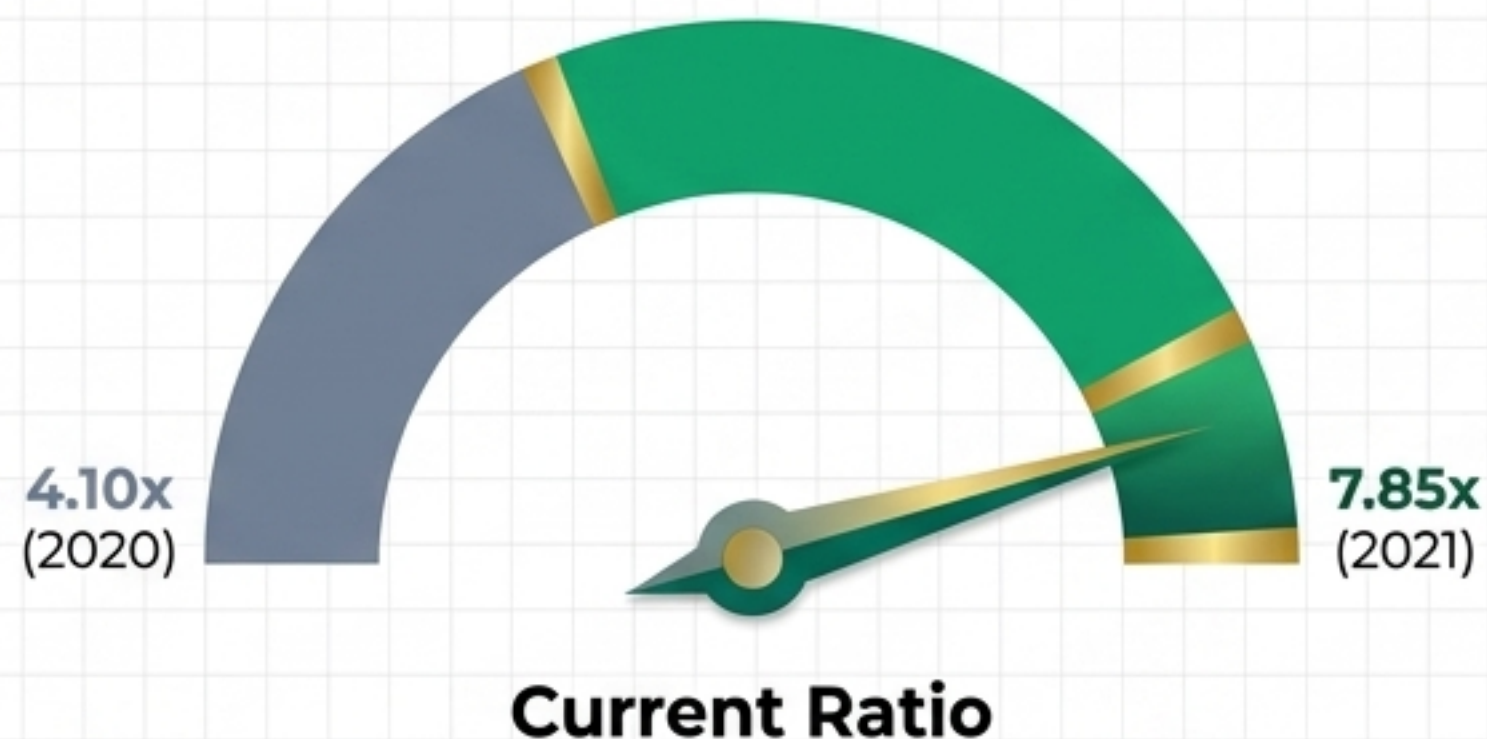
**Pillar 3:
Strategic Scaling**

Bypass physical clinic limits completely by introducing uncapped Pure Donation channels.

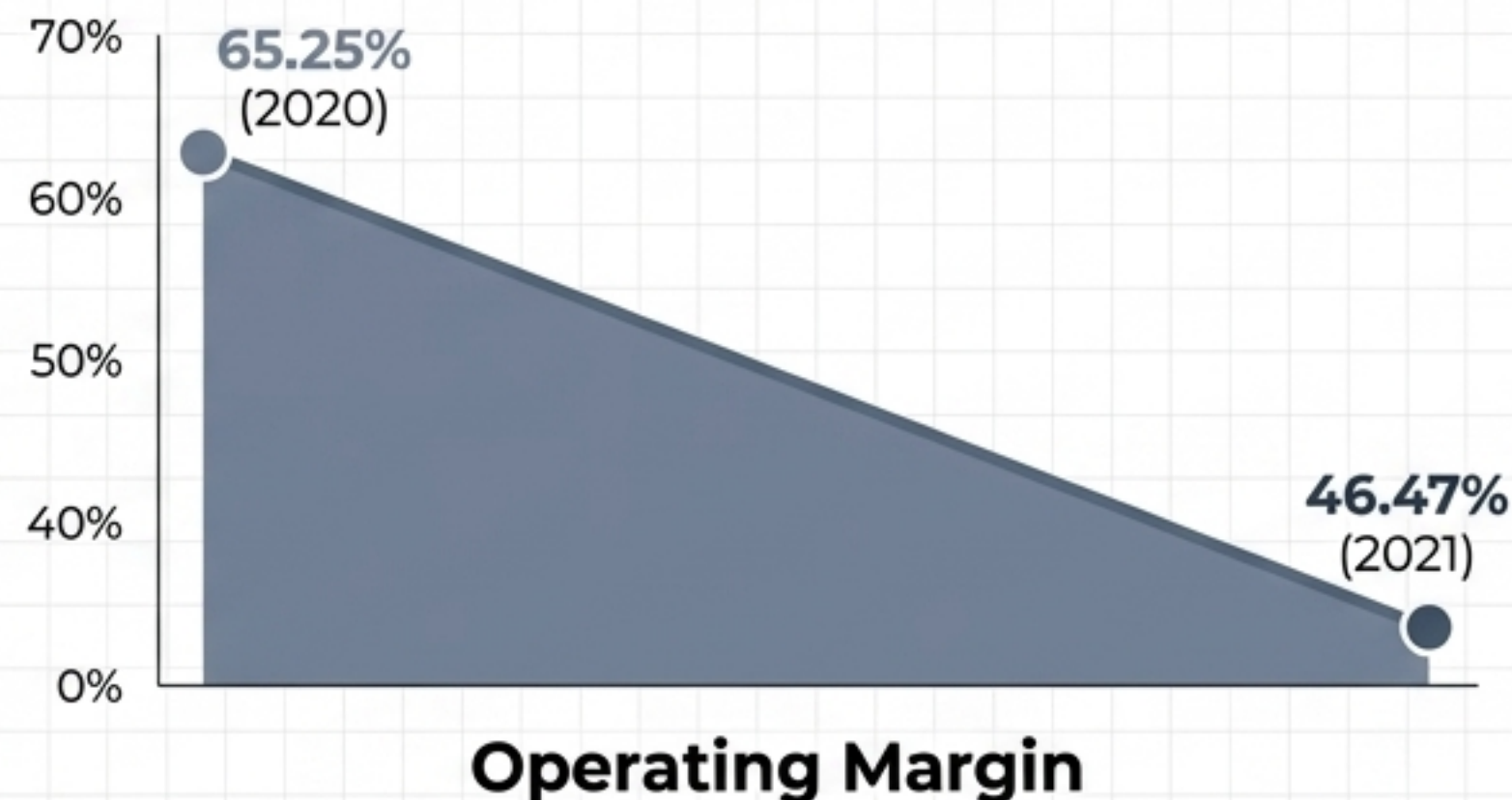
FINANCIAL HEALTH CHECK: IDENTIFYING STRUCTURAL IMBALANCES

The Need for Change

EXCESS IDLE LIQUIDITY



OVERHEAD PRESSURE



Liquidity vs. Efficiency

While total income grew from \$400k to \$850k year-over-year, our operational overhead is growing faster than revenue.

The Opportunity

The 7.85x current ratio indicates we are hoarding cash. We must deploy this war chest into a lean, high-margin event to reverse our declining operating margins.

Activating Our Internal War Chest

Capital Allocation Strategy

Zero External Debt

By deploying just \$59,000 of our \$284,000 in idle bank cash, we fully fund the event internally.

Risk Profile

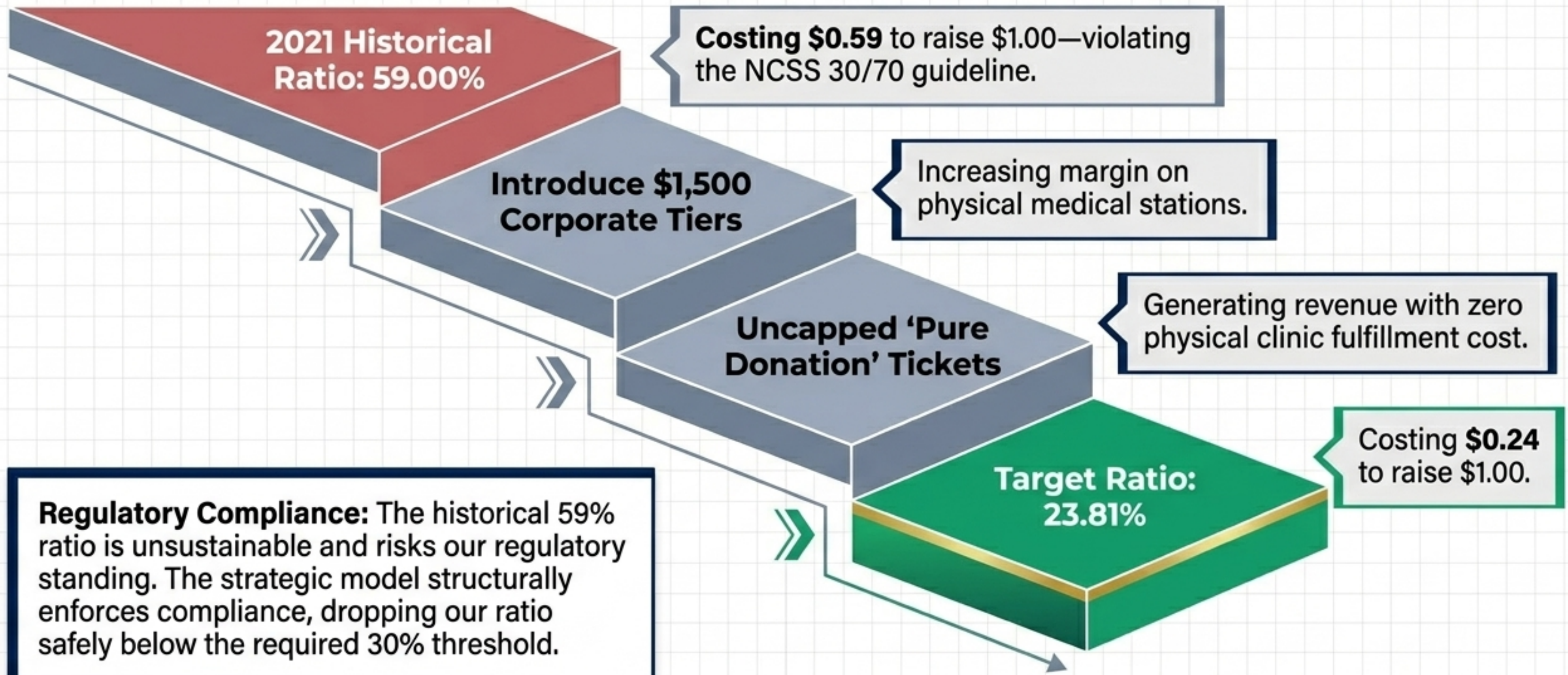
This leverages our inefficient 7.85x Current Ratio without impacting day-to-day liquidity, completely avoiding the need for outside loans or commercial debt.

\$284,000 Current Bank Cash



Recovering Fundraising Efficiency

Fixing the Fundraising Cost Ratio



Strategic Risk Mitigation & CSR Integration

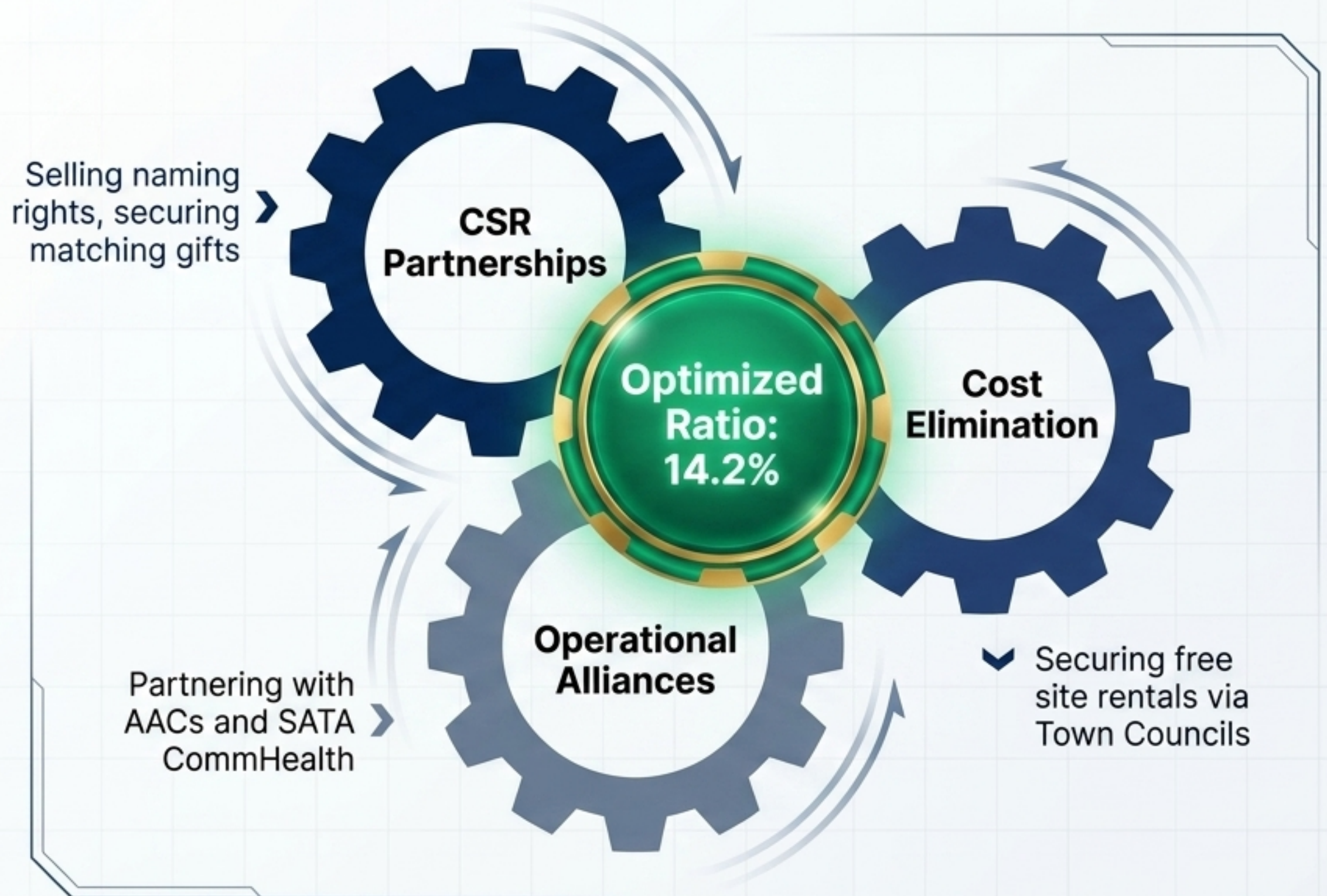
Expanding Margins through Ecosystem Partnerships

Accelerating Margins

By leveraging the local health ecosystem, we can eliminate the projected \$30,000 Site Rental (including set-up) fixed cost entirely.

The Result

Removing this \$30,000 burden drives our efficiency ratio down to an elite 14.2%, maximizing the impact of every donor dollar for our needy patients.



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Financial Projections: Default vs. Strategic

Scenario A vs. Scenario B

	Scenario A (Default Cap)	Scenario B (Strategic Model)
Fixed Costs	\$59,000	\$59,000
Gross Revenue	\$126,000	\$270,000
Efficiency Ratio	>50%	<24%
Net Surplus	\$61,720 (Fails Target)	\$205,720 (Meets Target)

The Mathematical Reality: The default scenario mathematically guarantees failure due to physical bottlenecks. The strategic model locks in the exact same fixed costs (\$59k) while unlocking massive upside via uncapped corporate and pure donation tiers.

Operational Optimization: Preventing Financial Leakage

Protecting Our Margins



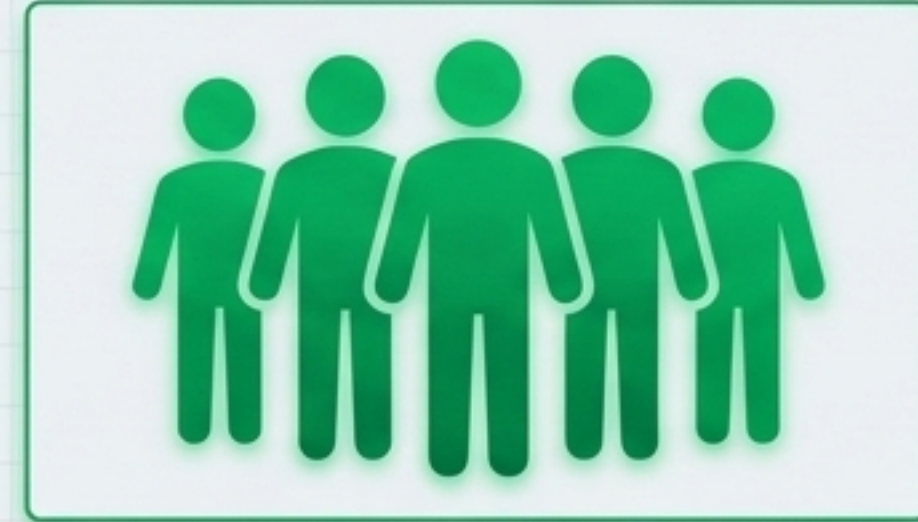
Paid Registration Worker (\$60/day)



Paid General Worker (\$50/day)



Core Volunteer Deployment



5 regular volunteers
replace paid staff

Freezing Overhead

To protect our operating margin and guarantee our \$205k net, we are enacting a rigid freeze on variable labor costs.

Resource Reallocation

We will deploy our existing core of 5 regular volunteers to completely replace paid administrative and general event staff, keeping our operating margins protected.

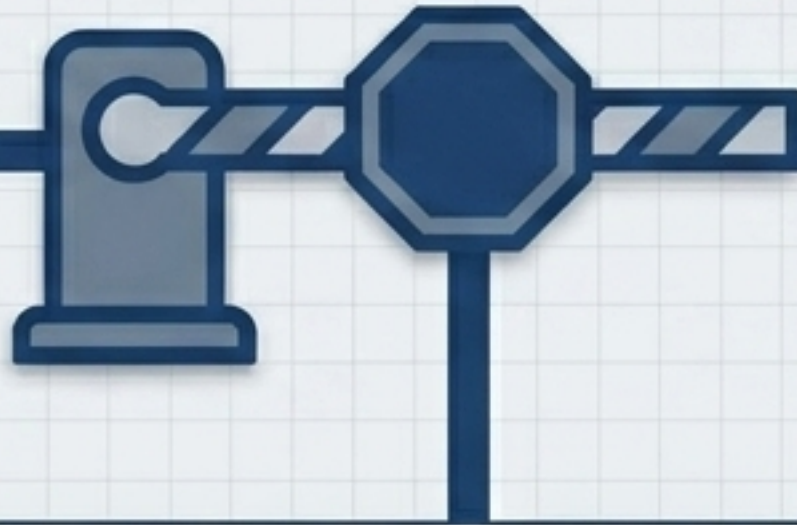
Risk Management Controls: Protecting the Downside

Strict Operational Governance

Control Point 1: 14-Day Go/No-Go Gate



Control Point 2: Portal Time Slots



Hard Stop Evaluation

We have established a rigid 14-day Go/No-Go gate. If registration numbers do not mathematically support our minimum viable net, we halt execution before final capital is irreversibly deployed.



Crowd & Capacity Management

To manage the physical clinic constraint of 250 people, we are instituting staggered registration time slots to prevent medical station bottlenecks.

Launching the Strategic Realignment

Board Approval Request

The Ask: We have the data, the financial modeling, and the strict operational controls in place. We request the Board's formal approval to activate this blueprint.



1. **Capital:** Authorize the deployment of **\$59,000** from current cash reserves to cover all upfront fixed costs.



2. **Pricing:** Approve the implementation of the **\$1,500** Corporate Ticket Tier and the uncapped Pure Donation model.



3. **Partnerships:** Greenlight the immediate initiation of CSR ecosystem partnerships (Town Councils, AACs, SATA CommHealth) to drive our efficiency ratio to **14.2%**.